

24CHCV04253 24CHCV04253

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Case Number: 24CHCV04253 Hearing Date: July 13, 2026 Dept: F49

Dept. F49

Date: 7/13/26

Case Name: Nana Dolidze v. Babak Eshaghian, M.D.; PulmoCrit

Associates; Robert Thompson, M.D.; Regal Medical Group; Robert Krasny, M.D.;

Liberty Pacific Advanced Imaging; and Does 1-50

Case No. 24CHCV04253

LOS ANGELES SUPERIOR COURT

NORTH VALLEY DISTRICT

DEPARTMENT F49

JULY 13, 2026

MOTION FOR SUMMARY

JUDGMENT, OR IN THE ALTERNATIVE, SUMMARY ADJUDICATION

Los Angeles Superior

Court Case No. 24CHCV04253

Motion filed: 4/3/26

MOVING PARTY: Defendants Babak Eshaghian, M.D. and

Sani and Eshaghian, M.D., Inc. dba PulmoCrit Associates

RESPONDING PARTY: Plaintiff Nana Dolidze

NOTICE: OK

RELIEF REQUESTED: An order from this Court granting Defendants Babak Eshaghian, M.D. and Sani and Eshaghian, M.D., Inc. dba PulmoCrit Associates' motion for summary judgment, or in the alternative, summary adjudication as to Plaintiff's Complaint.

TENTATIVE RULING: The Motion is GRANTED.

BACKGROUND

This is a medical malpractice action arising from injuries allegedly sustained by Plaintiff Nana Dolidze ("Plaintiff") due to negligent examination, diagnosis, and treatment of a lung nodule that appeared on Plaintiff's medical imagery. On November 18, 2024, Plaintiff filed the Complaint against Defendants Babak Eshaghian, M.D., ("Eshaghian"), Sani and Eshaghian, M.D., Inc. dba PulmoCrit Associates ("S&E, Inc."), Robert Thompson, M.D. ("Thompson"), Regal Medical Group ("Regal"), Robert Krasny, M.D. ("Krasny"), Beverly Radiology Medical Group, III ("Beverly," erroneously sued as "Liberty Pacific Advanced Imaging"), and Does 1 through 50 (collectively, "Defendants"), alleging a single cause of action for medical malpractice/wrongful death. On January 29, 2025 and February 24, 2025, Plaintiff filed amendments to the Complaint to name Northridge Diagnostic Imaging Center – A Radnet Imaging Center ("Northridge") as Doe 11 and Estate of Matthew Charms, M.D. ("Estate") as Doe 2, respectively. On October 8, 2025, at the request of Plaintiff, Regal was dismissed from the action.

On January 21, 2025, Eshaghian, S&E, Inc., and Thompson filed their joint Answer to the Complaint.

On January 31, 2025, Krasny and Beverly filed their joint Answer.

On March 2, 2025, Northridge and Estate filed their joint Answer.

On

April 3, 2026, Eshaghian and S&E, Inc. (collectively, "Moving Defendants") filed the instant Motion for Summary Judgment, or in the Alternative, Summary Adjudication (the "Motion").

On June 23, 2026, Plaintiff filed an Opposition.

On July 2, 2026, Moving Defendants filed a Reply.

ANALYSIS

"A

party may move for summary judgment in an action or proceeding if it is contended that the action has no merit or that there is no defense to the action or proceeding." (Code Civ. Proc., § 437c(a)(1).)

"The

motion for summary judgment shall be granted if all the papers submitted show that there is no triable issue as to any material fact and that the moving party is entitled to a judgment as a matter of law. In determining if the papers show that there is no triable issue as to any material fact, the court shall consider all of the evidence set forth in the papers, except the evidence to which objections have been made and sustained by the court, and all inferences reasonably deducible from the evidence, except summary judgment shall not be granted by the court based on inferences reasonably deducible from the evidence if contradicted by other inferences or evidence that raise a triable issue as to any material fact." (Code Civ. Proc., § 437c, subd. (c).)¿

A

party may alternatively seek summary adjudication on whether a cause of action, affirmative defense, or punitive damages claim has merit or whether a defendant owed a duty to a plaintiff. (Code Civ. Proc., § 437c, subd. (f)(1).) "A motion for summary adjudication ... shall proceed in all procedural respects as a motion for summary judgment." (Code Civ. Proc., § 437c, subd. (f)(2).)

The

moving party "bears an initial burden of production to make a prima facie

showing of the nonexistence of any triable issue of material fact.” (Aguilar v. Atlantic Richfield Co. (2001) 25 Cal.4th 826, 850.) To meet that burden, the moving party must present evidence sufficient to show they are entitled to summary judgment as a matter of law. (Code Civ. Proc., § 437c, subds. (c) & (f)(2); Monticello Ins. Co. v. Essex Ins. Co. (2008) 162 Cal.App.4th 1376, 1385.)

“In order to prevail on a motion for summary judgment, a defendant must either disprove an essential element of the plaintiff’s cause of action or prove an affirmative defense that would bar such cause of action.” (Velasquez v. Truck Insurance Exchange (1991) 1 Cal.App.4th 712, 717.)

The pleadings serve as the “outer measure of materiality” in a summary judgment motion, delimiting the scope of the issues that may be triable. (FPI Development, Inc. v. Nakashima (1991), 231 Cal.App.3d 367, 381.)

A.

Evidentiary Objections

Moving Defendants object to the declarations of Kevin Shaw, M.D. (“Dr. Shaw”) and Erik Peterson (“Peterson”), submitted by Plaintiff in support of her Opposition, in their entirety on the ground that they are not signed under penalty of perjury and constitute hearsay.

Pursuant to Code of Civil Procedure section 437c, subdivision (b)(2), an opposition to a summary judgment motion must, where appropriate, “consist of affidavits, declarations, admissions, answers to interrogatories, depositions, and matters of which judicial notice shall or may be taken.”

Code of Civil Procedure section 2015.5 requires that an unsworn declaration include a recitation “that it is certified or declared ... to be true under penalty of perjury,” be subscribed by the declarant, and state the date and place of execution. “A declaration not signed under penalty of perjury under the laws of California has ‘no evidentiary value’ and can be disregarded.” (Safieddine v. MBC FZ, LLC (2024) 103 Cal.App.5th 1086, 1094, fn. 9, citing ViaView, Inc. v. Retzlaff (2016) 1 Cal.App.5th 198, 217.) Without the recitation that the declaration is made

under penalty of perjury under California law, a declaration is not sufficiently reliable to be admitted into evidence. (*Kulsrestha v. First Union Commercial Corp.* (2004) 33 Cal.4th 601, 611-612 [declaration inadmissible where there is no “express facial link to California or its perjury laws”].)

Defects under section 2015.5 are curable before the hearing on a motion for summary judgment. (E.g. *Gall v. Smith & Nephew, Inc.* (2021) 71 Cal.App.5th 117, 125 [finding no error in trial court's consideration of evidence attached to a declaration the defect of which was fixed before the hearing].)

Here, the declarations of Dr. Shaw and Peterson both state when and where they were executed, but neither includes a recitation that it was made under penalty of perjury. (See Shaw Decl., at p. 11; Peterson Decl., at p. 3.) Without acknowledgement that the declarations are made subject to California's perjury laws, the declarations are unreliable as evidence.

The Court accordingly SUSTAINS Moving Defendants' objection Nos. 1 and 5 to the declarations in their entirety. Moving Defendants' remaining objections to specific portions of the declarations are OVERRULED as moot.

B.

Motion for Summary Judgment

Plaintiff alleges a single cause of action against all Defendants for medical malpractice. To prevail on a cause of action for medical malpractice, Plaintiff must establish: (1) the duty of the professional to use such skill, prudence, and diligence as other members of his profession commonly possess and exercise; (2) breach of that duty; (3) proximate cause; and (4) actual loss or damage. (*Avivi v. Centro Medico Urgente Medical Center* (2008) 159 Cal.App.4th 463, 468, fn.2.)

Moving Defendants contend that Plaintiff cannot establish either that they breached the relevant standard of care or that their conduct caused Plaintiff's injuries. (Mot., at pp. 8-11.) The Court addresses each of these elements in turn.

(1)

Breach of the Standard of Care

“The law requires only that doctors exercise that reasonable degree of skill, knowledge and care ordinarily possessed and exercised by doctors under similar circumstances in diagnosis and treatment, with no different or higher degree of responsibility than that obtaining in their professional community.” (Folk v. Kirk (1975) 53 Cal.App.3d 176, 185.) “The standard of skill, knowledge and care prevailing in a medical community is ordinarily a matter within the knowledge of experts.” (Ibid.; Willard v. Hagemeister (1981) 121 Cal.App.3d 406, 412 [expert evidence is conclusive as to the propriety of the practitioner’s conduct under the circumstances unless the conduct required by the circumstances is within a layperson’s common knowledge].)

Moving Defendants bear the initial burden of producing evidence establishing that there is no triable issue as to breach.

In support of the Motion, Moving Defendants submit the declaration of Andrew Wachtel, M.D. (“Dr. Wachtel”), subscribed and signed under penalty of perjury in Los Angeles, California. Dr. Wachtel, a licensed physician board certified in internal medicine and pulmonary medicine, attests that his review of Plaintiff’s medical records and the deposition transcripts of Plaintiff, Eshaghian, and Ika Nastasin (“Nastasin”) lead him to conclude that Eshaghian and his staff appropriately adhered to the relevant standard of care, including Fleischner guidelines. (Wachtel Decl., ¶¶ 1, 3, 5.) Fleischner guidelines are criteria made by the Fleischner Society for recommendations for follow-up management of pulmonary nodules detected on individuals 35 years of age and older. (Id., ¶ 5c.) Dr. Wachtel attests that Eshaghian’s examination and follow-up management of Plaintiff complied with both the Fleischner guidelines and general standard of care. (Id., ¶ 5a-5d.)

Wachtel’s finding is based on the following consultations and scans Plaintiff underwent between March 25, 2021 and March 12, 2024:

(1)

March 25, 2021: Eshaghian’s office first consulted with

Plaintiff upon referral from her primary care provider. The record of this visit noted that Plaintiff's lung nodule was stable and she was instructed to follow up in two to three months with CT and lab results. (Wachtel Decl., ¶ 4d; Undisputed Material Fact ("UMF") No. 5; Metros Decl., Exh. C, Bates Nos. 0012-0014.)

(2)

April 16, 2021: Plaintiff's next chest CT, ordered by Eshaghian, was performed. The imaging showed no significant change in the nodule, which was noted as being "spiculated," or having pointy edges. (Wachtel Decl., ¶ 4e; UMF No. 6; Metros Decl., Exh. B, Bates Nos. 00018-00019.)

(3)

June 10, 2021: Plaintiff presented for a follow-up where Plaintiff's November 2020 PET scan, January 2021 chest CT, and April 2021 chest CT were reviewed. A plan was formulated for Plaintiff to undergo another scan and pulmonary function test in October 2021. (Wachtel Decl., ¶ 4f; UMF No. 7; Metros Decl., Exh. C, Bates Nos. 0018-0020.)

(4)

October 7, 2021: Charms performed and interpreted a follow-up chest CT, ordered by Eshaghian. The record of this visit notes no change in Plaintiff's lung nodule from previous scans, no new nodules, and no "ground glass infiltrates" that would suggest a tumor or pneumonia. (Wachtel Decl., ¶ 4g; Metros Decl., Exh. D.)

(5)

October 21, 2021. Plaintiff presented for a physical exam at Eshaghian's office. The October 7, 2021 chest CT was reviewed. The visit notes indicate that the nodule had been stable in size since November 2020, and Plaintiff's pulmonary function test showed no obstruction or restriction and normal diffusion. Plaintiff was presented with the option to undergo a lung biopsy or continue serial imaging, and elected to continue with serial imaging. (Wachtel Decl., ¶ 4h-4i; Metros Decl., Exh. C, Bates Nos. 0023-0025.)

(6)

March 1, 2022: Plaintiff underwent another chest CT ordered by Eshaghian. Reza Hayeri, M.D. noted no significant change in the lung nodule. (Wachtel Decl., ¶ 4j; Metros Decl., Exh. B, Bates Nos. 00016-00017.)

(7)

May 12, 2022: Plaintiff presented at Eshaghian's office for consultation, reporting that she was doing well and denying any respiratory symptoms. (Wachtel Decl., ¶ 4k; UMF No. 12; Metros Decl., Exh. C, Bates Nos. 0026-0027.)

(8)

October 6, 2022: Plaintiff failed to appear for an appointment at Eshaghian's office. (Wachtel Decl., ¶ 4k; UMF No. 12.)

(9)

December 1 2022: Plaintiff underwent another chest CT. Krasny interpreted the CT and noted that the lung nodule was similar to the prior scans but possibly enlarged compared to September 2020. Krasny recommended a follow-up in 12 months. (Wachtel Decl., ¶ 4l; UMF No. 13; Metros Decl., Exh. B, Bates Nos. 00013-00014.)

(10)

November 21, 2023: Plaintiff underwent a chest CT, which revealed that the lung nodule had grown. The interpreting radiologist listed several diagnostic considerations and indicated that he would order a CT-guided biopsy and also recommendation a PET scan. (Wachtel Decl., ¶ 4n; UMF No. 15; Metros Decl., Exh. B, Bates Nos. 00006-00007.)

(11)

December 12, 2023: Plaintiff presented to Eshaghian and reported that she was doing well since her last visit and had no respiratory issues. Eshaghian performed a physical exam and noted that Plaintiff's lungs were clear and she had no labored breathing. (Wachtel Decl., ¶ 4o; UMF No. 16; Metros Decl., Exh. C., Bates Nos. 0028-0029.)

(12)

January 9, 2024: Plaintiff presented to Eshaghian, who advised that insurance authorization had not yet been provided for a biopsy and referred Plaintiff to undergo a CT-guided lung nodule biopsy and PET scan. (Wachtel Decl., ¶ 4p; UMF No. 17; Metros Decl. Exh. C, Bates Nos. 0030-0031.)

(13)

January 30, 2024: Liberty Imaging performed a PET scan

that revealed a hypermetabolic nodule and a hypermetabolic pre-vascular lymph node, both of increased size compared to earlier imaging. (Wachtel Decl., ¶ 4q; UMF No. 18; Metros Decl., Exh. B, Bates Nos. 00004-00005.)

(14)

February 5, 2024: Plaintiff underwent a biopsy at Providence Holy Cross and the results came back positive for pulmonary adenocarcinoma on February 19, 2024. (Wachtel Decl., ¶ 4r; UMF No. 19; Metros Decl., Exh. F, Bates Nos. 9, 17-50.)

(15)

February 20, 2024: Eshaghian referred Plaintiff to cardiothoracic surgeon Anthony Kim, M.D. (“Dr. Kim”) and medical oncologist Benjamin Leach, M.D. They later confirmed the finding of lung cancer, but Dr. Kim discontinued surgery due to evidence that the cancer had spread. Dr. Kim recommended following up with oncology to proceed with immunotherapy. (Wachtel Decl., ¶ 4s-4t; UMF Nos. 20-21; Metros Decl., Exh. C, Bates Nos. 0035-0037.)

(16)

March 12, 2024: Plaintiff consulted Eshaghian, denying any wheezing, coughing, or shortness of breath. Eshaghian reviewed Plaintiff’s chart and prior imaging and discussed with Plaintiff and her family the prior decision to proceed with CT monitoring over a biopsy. (Wachtel Decl., ¶ 4u; UMF No. 22; Metros Decl., Exh. C, Bates Nos. 0032-0034.)

Dr. Wachtel attests that the foregoing consultations and imaging show that Eshaghian and his staff had Plaintiff undergo imaging approximately every six months to monitor the nodule. (Wachtel Decl., ¶ 6.) This monitoring, Dr. Wachtel concludes, was appropriate given the lack of change in the size of the nodule between March 2021 and May 2022—before Plaintiff’s health insurance provider changed and she began seeing a different pulmonologist. (Id., ¶¶ 4m, 5d.) Dr. Wachtel attests that the monitoring ordered by Eshaghian was sufficient and a biopsy was not required under the Fleischner guidelines because the three CT scans performed on April 16, 2021, October 7, 2021, and March 1, 2022 did not reveal any concerning changes that would have necessitated additional evaluative methods. (Id., ¶ 5d.) The imaging that revealed significant changes occurred after Plaintiff stopped seeing Eshaghian, and once Plaintiff re-established care with Eshaghian, he promptly ordered a biopsy and PET scan due to the growth of the nodule. (Wachtel Decl., ¶ 5e.)

Based on the foregoing, the Court finds that Moving Defendants have met their burden of producing evidence that Eshaghian and his office staff complied with the relevant standard of care for the examination and diagnosis of Plaintiff's condition. The burden accordingly shifts to Plaintiff to produce evidence establishing a triable issue of material fact as to breach.

Plaintiff posits that the relevant standard of care required Eshaghian to recommend a biopsy when Plaintiff first presented to him on March 25, 2021, and at every consultation thereafter, because of the changes in the lung nodule from prior imaging. (Opp'n, at pp. 4-6.) Plaintiff contends that his failure to do so constituted breach. (Ibid.) Plaintiff, however, fails to present admissible evidence establishing a standard of care other than the standard to which Dr. Wachtel attests. As noted above, Plaintiff's expert, Dr. Shaw, does not make his declaration under penalty of perjury, and it therefore has no evidentiary value. As to the relevant standard of care and breach, therefore, Plaintiff has failed to produce evidence raising a triable issue.

(2)

Causation

Moving Defendants also contend that Plaintiff cannot raise a triable issue as to causation.

"[C]ausation must be proven within a reasonable medical probability based upon competent expert testimony." (Dumas v. Cooney (1991) 235 Cal.App.3d 1593, 1603, internal quotations omitted.)

Dr. Wachtel attests that "[n]othing that Dr. Eshaghian or Sani and Eshaghian, M.D., Inc. did, or did not do, caused and/or contributed to any of plaintiff's alleged injuries in this case." (Wachtel Decl., ¶ 5f.) Plaintiff contends that this bare conclusion, without any analysis in support, is insufficient to meet Moving Defendants' burden. The Court agrees.

An expert declaration that merely asserts ultimate facts and conclusions, without factual detail and a reasoned explanation, cannot satisfy a defendant's burden of showing the absence of a

genuine issue of material fact. (Zarazoga v. Adam (2025) 109 Cal.App.5th 113, 119.)

Here, as Plaintiff notes, Dr. Wachtel does not indicate whether an earlier biopsy would have revealed a less progressed stage of cancer or explain how earlier diagnosis would not have altered Plaintiff's ultimate outcome. As to causation, the Court thus finds that Moving Defendants have failed to meet their initial burden.

Nevertheless, because the Court has found that Moving Defendants have produced evidence negating breach, and Plaintiff has not met her burden of raising a triable issue as to breach, the Court GRANTS the Motion.

CONCLUSION

The Court GRANTS Defendants Babak Eshaghian, M.D. and Sani and Eshaghian, M.D., Inc. dba PulmoCrit Associates' motion for summary judgment.

Moving party to give notice.